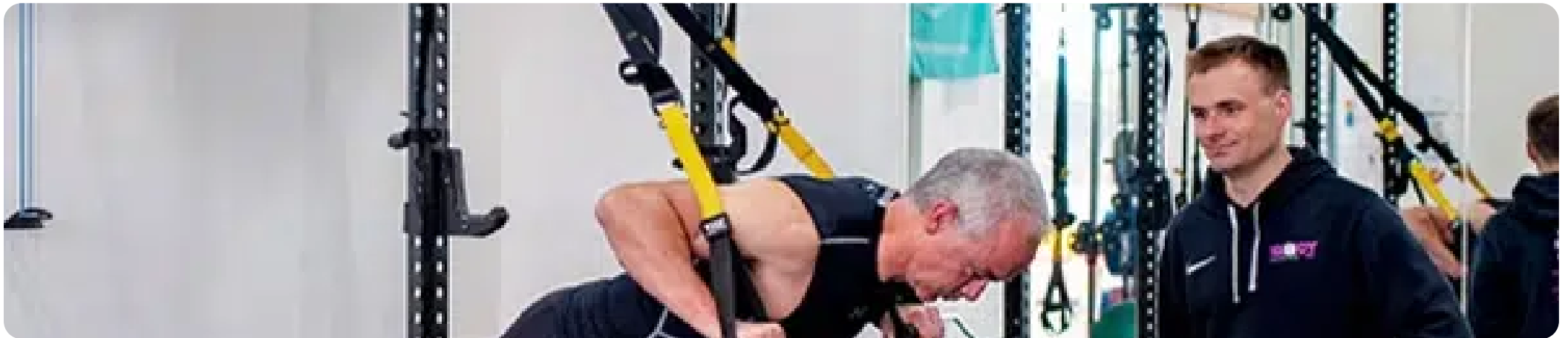




LINKEDIN CONTENT PACK · TARA'S PROFILE

Month 3 — LinkedIn

A full month of referrer-focused LinkedIn content spotlighting our exercise physiology service — a weekly clinical newsletter plus two posts each week, positioning Your Body Hub as the trusted exercise-physiology referral partner for GPs and NDIS coordinators in Officer and the south-east.



Newsletters ×4 (weekly)

Posts ×8

Audience: GPs & coordinators

Officer & south-east

Prepared by Ready to Rank · Draft for client sign-off

THE GOAL OF THIS PACK

Position Your Body Hub as the trusted exercise-physiology referral partner for local GPs and NDIS coordinators. The newsletter shows how our AEP thinks and works; the posts distribute it and build the relationship. Lead with the referrer's problem (patients told to "get more active" with no safe plan) and the referral pathway — not a sales pitch.

■ Weekly newsletter ■ LinkedIn post (Tara)

Cadence: 1 newsletter + 2 posts per week — Tuesday (newsletter) · Wednesday & Friday (posts)

Monday	Tuesday	Wednesday	Thursday	Friday	Sat & Sun
31 Aug	Week 1 · Falls Prevention 1 NEWSLETTER Falls prevention: the referral that keeps patients independent	POST 1 2 Newsletter lead — strength & balance cuts falls risk	3	POST 2 4 When to refer for falls & balance	5 · 6
7	Week 2 · Chronic Disease 8 NEWSLETTER Exercise as medicine: the CDM pathway	POST 1 9 CDM & the GP Management Plan pathway	10	POST 2 11 Joined-up care under one roof	12 · 13
14	Week 3 · Pain, Fatigue & Cancer 15 NEWSLETTER Graded exercise for persistent pain & cancer recovery	POST 1 16 Why rest backfires for persistent pain	17	POST 2 18 Exercise oncology & survivorship	19 · 20
21	Week 4 · NDIS & Complex 22 NEWSLETTER Exercise physiology & NDIS: goal-focused support	POST 1 23 For NDIS coordinators	24	POST 2 25 Month recap + next month (Myotherapy)	26 · 27
28	29	30	1 Oct	2 Oct	3 · 4

Note: A weekly newsletter keeps the exercise-physiology service front-of-mind with referrers; the two posts extend reach and prompt connection. Dates are the recommended cadence and can shift to suit.

NEWSLETTER · TUE 1 SEP

Falls prevention: the referral that keeps your patients independent

A WEEKLY CLINICAL NOTE FROM TARA · YOUR BODY HUB, OFFICER

Every GP has older patients quietly losing confidence on their feet — and a fall is often the event that ends independence, or starts a hospital admission.

The evidence is consistent: targeted strength and balance training measurably reduces falls risk. The key word is *targeted* — a "stay active" brochure rarely shifts the outcome.

Our Accredited Exercise Physiologist, James, assesses each patient and prescribes a progressive, supervised program — the kind of structured intervention that actually changes function. Accessible under Medicare care plans, NDIS and private health.

For your older patients — especially post-fall, or newly unsteady — earlier referral means more function retained.

Subscribe for a weekly clinical note — and refer through anytime. — Tara, Your Body Hub, Officer · **CTA: [Subscribe](#)**

Post 1 · Wed 2 Sep · newsletter lead

✉ This week's clinical note: **the falls-prevention referral that keeps patients independent.**
Targeted strength and balance training measurably reduces falls risk — far more than "stay active" advice. Here's how our AEP prescribes and progresses it.
If you look after older patients, this one's for you. 📧 Read & subscribe — link in comments.
— Tara, Your Body Hub, Officer · **CTA: [Subscribe](#)**

Post 2 · Fri 4 Sep · referral guidance

For GPs: when to refer for falls & balance.
A recent fall, a near-miss, new unsteadiness, or reduced confidence getting around — all worth a referral before function is lost. A supervised, progressive program beats a patient guessing alone.
James is an Accredited Exercise Physiologist — happy to take those referrals under a care plan, NDIS or privately.
— Tara, Your Body Hub, Officer · **CTA: [Refer / Connect](#)**

Week 1 · Notes & Feedback

Type your changes below, then tick the week off.

SIGN-OFF: Approved as is Approved with the changes above Needs another look

NEWSLETTER · TUE 8 SEP

Exercise as medicine: the GP Management Plan pathway

A WEEKLY CLINICAL NOTE FROM TARA · YOUR BODY HUB, OFFICER

For patients with type 2 diabetes, cardiovascular risk, obesity or other chronic conditions, exercise is genuinely first-line therapy — but "you should get more active" rarely lands on its own.

Under a GP Management Plan, your patients can access subsidised exercise-physiology sessions with James, who translates the clinical goal into a safe, specific, progressive program — and reports measurable change back to you.

Because our exercise physiology, dietetics and physiotherapy teams work under one roof, movement, nutrition and medical care pull in the same direction.

For your complex and chronic patients, that joined-up pathway is where we add the most value.

Subscribe for a weekly clinical note — and CDM referrals are always welcome. — Tara, Your Body Hub, Officer · CTA: [Subscribe](#)

Post 1 · Wed 9 Sep · CDM pathway

Exercise is first-line in chronic disease — but "get more active" rarely lands alone.

Under a GP Management Plan, your patients get subsidised exercise-physiology sessions and a safe, specific plan they'll actually follow. CDM referrals welcome.

This week's note has the pathway. 📄 link in comments.

— Tara, Your Body Hub, Officer · CTA: [Refer](#)

Post 2 · Fri 11 Sep · joined-up care

Joined-up care: our exercise physiology, dietetics and physio teams work under one roof.

For complex or chronic patients, that means movement, nutrition and medical care pulling in the same direction — and one point of contact for you, with clear reporting back.

— Tara, Your Body Hub, Officer · CTA: [Connect](#)

Week 2 · Notes & Feedback

Type your changes below, then tick the week off.

SIGN-OFF:

Approved as is

Approved with the changes above

Needs another look

NEWSLETTER · TUE 15 SEP

Graded exercise for persistent pain, fatigue & cancer recovery

A WEEKLY CLINICAL NOTE FROM TARA · YOUR BODY HUB, OFFICER

Two groups of patients often fall through the gap: those with persistent pain who've been told to rest, and those recovering from — or going through — cancer treatment, exhausted and deconditioned.

For both, the evidence points the same way: carefully graded exercise helps. In persistent pain, graded exposure reduces sensitivity and rebuilds capacity. In cancer care, supervised exercise can ease fatigue and protect strength and function.

James, our Accredited Exercise Physiologist, works within scope and alongside the treating team — starting well within limits and progressing at the patient's pace.

For patients where "just rest" hasn't worked, or who need to move safely through treatment, this is a referral worth making early.

Subscribe for a weekly clinical note — and reach out to discuss a patient anytime. — Tara, Your Body Hub, Officer · CTA: [Subscribe](#)

Post 1 · Wed 16 Sep · persistent pain

For persistent pain, complete rest often backfires — the system gets more sensitive, not less. Graded exercise gently retrains the nervous system to feel safe moving again. Our AEP delivers it supervised and progressive. This week's clinical note goes deeper. 📄 link in comments. — Tara, Your Body Hub, Officer · CTA: [Subscribe](#)

Post 2 · Fri 18 Sep · exercise oncology

Exercise oncology: supervised movement through and after cancer treatment. Tailored exercise can help ease fatigue and protect strength and function — delivered within scope and alongside the treating team. If you're caring for a patient through treatment or into survivorship, we're happy to help. — Tara, Your Body Hub, Officer · CTA: [Refer / Connect](#)

⚠️ **Compliance:** exercise oncology & persistent-pain content is evidence-based but general — no cure/outcome claims; frame as "within scope, alongside the treating team". Programs individually assessed & supervised.

Week 3 · Notes & Feedback

Type your changes below, then tick the week off.

SIGN-OFF: Approved as is Approved with the changes above Needs another look

NEWSLETTER · TUE 22 SEP

Exercise physiology & NDIS: goal-focused support for complex patients

A WEEKLY CLINICAL NOTE FROM TARA · YOUR BODY HUB, OFFICER

For participants living with neurological conditions, disability or complex health needs, exercise physiology is one of the most practical, goal-aligned supports on a plan — and one that's often under-used.

James, our Accredited Exercise Physiologist, builds individually assessed, supervised programs around participant goals: mobility, strength, independence, falls reduction and wellbeing — and works alongside support coordinators, therapists and the wider allied-health team.

Two things worth flagging for referrers and coordinators: sessions are goal-focused and reportable, and we can coordinate with the participant's other supports under one roof.

If you're supporting someone who'd benefit, earlier engagement usually means better outcomes.

Subscribe for a weekly clinical note — and reach out about a participant anytime. — Tara, Your Body Hub, Officer · CTA: [Subscribe](#)

Post 1 · Wed 23 Sep · NDIS

For NDIS coordinators: exercise physiology that supports participant goals.

From mobility and strength to independence, falls reduction and wellbeing, James delivers practical, goal-focused, supervised sessions — assessed individually and reported clearly.

Happy to discuss a participant anytime.

— Tara, Your Body Hub, Officer · CTA: [Refer / Connect](#)

Post 2 · Fri 25 Sep · month recap

That's a wrap on our exercise-physiology series.

Thank you to the GPs and coordinators across Officer, Berwick and Pakenham who trust us with your patients and participants.

Next month we turn to Myotherapy — hands-on treatment for muscular pain and tension. Subscribe so you don't miss it.

— Tara, Your Body Hub, Officer · CTA: [Subscribe](#)

Week 4 · Notes & Feedback

Type your changes below, then tick the week off.

SIGN-OFF:

Approved as is

Approved with the changes above

Needs another look